

CLTC Area Office TEFRA Policy October 2023

1. Receive referral in CLTC **Centralized Intake** from Medicaid Eligibility TEFRA Unit. Cover letter will be attached.
2. If the applicant is enrolled in the Medically Complex Children's waiver, **do not open an application in Phoenix**. Centralized Intake will now release to the Area Offices TEFRA referrals when the child is participating in the MCC waiver. Area office staff will assign the TEFRA application to a nurse and then immediately assign the TEFRA application to the SCDHHS TEFRA Coordinator who will then forward the most recent MCC assessment to the Medicaid Eligibility TEFRA Unit and terminate the TEFRA application.
3. Centralized Intake processes referral with reason for referral as "TEFRA".
4. Centralized Intake releases the application to the CLTC Area Office for assignment to a **Nurse Consultant**.
5. **Nurse Consultant** will complete the assessment within **ten (10) business days** from the case assignment and determine the Level of Care **within three (3) business days** of the completion of the assessment. If the Level of Care completion is delayed, the reason for the delay must be clearly documented in the narrative. If a signed Consent Form is not received from the Medicaid Eligibility TEFRA Unit with the referral information, a signed Consent Form should be obtained by the Nurse Consultant.
Assessment setting can be telephonic, home, school, CLTC Area Office or other. If "other" is selected on the **Nurse Consultant** checklist, it must have Area Administrator approval, and the setting documented in the narrative.
If the Level of Care is questionable or unclear, a face-to-face visit **must** be made. If the applicant lives in a county not served by the assessing office and a face-to-face visit is necessary, notification to the Area Office Administrator and/or Lead Team Nurse Consultant in which the applicant resides will be notified via email and phone call so the receiving office may schedule the home visit. The case will be transferred via Phoenix to the receiving office. The transfer activity and receipt of the case must be documented in the narrative.
If the Level of Care remains unclear after assessment, the nurse completing the face-to-face visit will be responsible for sending the Physician Information Form and all steps clearly documented in the narrative.

6. If the **Nurse Consultant** is unable to contact the primary contact **within ten (10) business days**, a CLTC Notification Form, Form 171, should be mailed to the primary contact indicating the need to contact the CLTC office within 10 calendar days or the case will be closed as “No Response from Participant.” If the applicant is hospitalized, institutionalized or out of town and cannot be assessed, the **Nurse Consultant** should question the family as to how long the child is expected to be unavailable. If the child is expected to return home within 30 calendar days, the referral should remain open. If the child will be out of the home longer than 30 calendar days, the case should be closed as “Inappropriate after Intake.” The primary contact should be notified that an assessment cannot be completed and should contact the Medicaid Eligibility TEFRA Unit when the child is discharged or returns home. If the **Nurse Consultant** is unable to complete the assessment, notify the Medicaid Eligibility TEFRA Unit via email at Specialtyunit@scdhhs.gov using the TEFRA Level of Care Letter from the “forms” section of Phoenix to communicate this information.
7. **Nurse Consultant** will conduct the assessment using the South Carolina Level of Care for Medicaid Sponsored Long Term Care Services Manual, which includes the Guide to Developmental Stages of Children.
 - A. Please read page 16 of the manual carefully. This note addresses how functional deficits are to be coded. For example, if the one-year-old child cannot dress himself, this is age appropriate and would be coded total assistance with the comment that it is age appropriate that this child cannot dress himself.
 - B. For children under age twelve, completing of the psycho- behavioral section of the assessment is not required but comments should be made. Briefly describe the behavioral manifestations of daily habits or psychological problems. If a child under twelve demonstrates at least one clear deficit and significant behavior problems are observed by the **Nurse Consultant** or are documented by medical records, and are not age-appropriate, the behaviors should be coded appropriately, and the child may be determined to require intermediate service #4.
 - C. If a child under twelve demonstrates at least one clear deficit and medical records document significantly delayed cognitive functioning which is not age-appropriate, the cognitive section should be coded appropriately, and the child may be determined to require intermediate service #3.

8. Determine Level of Care utilizing South Carolina Assessment Level of Care Manual for Medicaid Sponsored Long Term Care Services criteria.

A. If the child meets Level of Care, the nurse will sign as the assessor and team staff with another nurse who will sign as the reviewer.

B. If the child does not appear to meet Level of Care, the Nurse Consultant will send the TEFRA Physician's Input Letter to the child's physician to request additional information. Once the TEFRA Physician's Input Letter has been completed and returned or 3 attempts within 10 business days has been given for return, the nurse will add any pertinent information to the assessment. If still unable to determine the Level of Care, the nurse will consult with the Lead Team Nurse Consultant and/or Area Administrator for review. If no Level of Care decision can be made, the case will be forwarded to the Central Office Designee for assistance.

C. If the child has a skilled need but no functional deficit, the Nurse Consultant will review with Lead Team Nurse and/or Area Administrator. The nurse will send the TEFRA Physician's Input Letter to the child's physician to request additional, current medical information about the child's condition. Once the TEFRA Physician's Input Letter has been completed and returned or 3 attempts within 10 business days has been given for return, the nurse will add any pertinent information to the assessment and forward any available medical information to the Central Office Designee for review by the Agency medical director.

NOTE: After review by the Agency medical director, the Central Office Designee will notify the Nurse Consultant with the Level of Care decision.

A. Scan all documents, letters, and copies of medical records in the applicant's record to appropriate Scan Tag under the Scans section in Phoenix.

9. After the Level of Care is established, the Nurse Consultant should determine if it is appropriate for the child to receive services outside of an institution **regardless of Level of Care determination.**

10. Complete the following **required** fields on the TEFRA Level of Care Letter:

- A. Assessment completion
- B. Nursing Home Level of Care
- C. Child appears to be appropriate to receive services in a community setting.

NOTE: The services may include but are not limited to speech therapy, occupational therapy, physical therapy, behavioral services, and educational services. Community settings may include but are not limited to home, school, outpatient facilities/clinics.

In **rare** circumstances, a child's special service needs may only be met in an institutional setting in which case the nurse will do the following:

- Select "No" for "Child appears to be appropriate to receive services in a community setting" on the TEFRA Level of Care Letter.
- Report findings to Lead Team Nurse Consultant and/or Area Administrator
- Document the decision in the narrative with supporting reasons
Example: "Child is deemed inappropriate for services in the community due to bedbound status".

11. Send the completed TEFRA Level of Care Letter to the address on the form or via email to the Medicaid Eligibility TEFRA Unit at Specialtyunit@scdhhs.gov.

NOTE: If the child is Medically Ineligible, include a copy of the assessment and any physician/medical documentation with this correspondence.

12. Close case to include the following reasons:

- A. If the child meets Level of Care, close as "TEFRA."
- B. If the child does not meet Level of Care, close as "Medically Ineligible."

13. If the primary contact calls with questions about an assessment, the requested information may be given verbally; **however, DO NOT SEND TEFRA LEVEL OF CARE LETTER TO PRIMARY CONTACT OR OTHER AGENCIES.**

